



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : امانى محمد حسين محمد
المحافظة : القاهرة الحي / المدينة : السيده زينب
رقم البطاقة : 26809230104044 رقم التليفون : 01000238168
العمر (تاريخ الميلاد) : 23-09-1968 المهنة : ربه منزل
العنوان : السيده زينب
تاريخ تسجيل الإستمارة : 19-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر بالصبغه
قسم العلاج :
الأورام

[11127]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
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التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: امانى محمد حسين محمد National ID: 26809230104044	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest and abdomen
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بمطابق قانون المصارف

أهلى
محمد حسين محمد الصغير
٢٢ درب النيلون
السيدة زينب - القاهرة

٢٠٨٠٩٢٣٠١٠٦٠٤٤

٢٠٨٠٩٢٣٠١٠٦٠٤٤

JU3204716

٢٠٢٣/٢ ٢٠٨٠٩٢٣٠١٠٦٠٤٤
بنون عسل

أهلى
عصام الدين عبدالرحمن عبداللطيف المصري
٢٠٣٠/٣/٢٠٦

البطاقة سارية حتى ٢٠٣٠/٣/٢٠٦

Patient Name	: الأستاذة / اماني محمد حسين	Request No.	: 13007808
Patient No.	: 26809230104044	Request Date	: 24/10/2023
Path No.	: 3687/23	Verification Date	: 13/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - عياد
Age	: 55 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 24/10/2023, 11:35am.

Gross Examination:

- Multiple grayish white tissue cores up to 1cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No .
- * Perineural invasion: No.
- * Angioinvasion: No.

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma , Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by :

Dr.Marwa mahmoud

Printing Date : 14/11/2023 13:38:50

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Printing By : marwa mohamed youss





Patient Name	: الأستاذ / أماني محمد حسين	Request No.	: 13007808
Patient No.	: 26809230104044	Request Date	: 24/10/2023
Ref No.	: 3687/23	Verification Date	: 13/11/2023
Sex	: Female	Financial Category	: اوزام دار السلام - مبلّره
Age	: 55 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Specimen: Tru-cut biopsy, collected at 24/10/2023, 11:35am.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

ER (Positive control showed appropriate reactivity):

-Cadherin : Positive .

ER: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8).

PR: Strong nuclear reaction in 30% of tumor cells (Allred score 6/8).

Her2: No membrane reaction in the tumor cells (score 0).

Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma , Grade II.

ER: Positive (score 7/8).

PR: Positive (score 6/8).

Her2: Negative (score 0).

Ki67: Positive in 10% of tumor cells.

Scoring system: "ER, PR"

CO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

Dr.Marwa mahmoud

Printing Date : 14/11/2023 13:38:43

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Printed By : .marwa mohamed yousse



????, ????? Patient ID: 10110225009606 DOB: 23-Sep-1968 Date: 31-May-2025

Patient Lastname: محمد

Patient Firstname: أماني

HIS Patient ID: 10110225009606

RIS Patient ID: 370354

Date of Birth: 23/09/1968

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (31/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" Small focal tracer uptake is noted at the left aspect DV9/10?? Marked degenerative changes (Facet joint arthropathy) for CT correlation.

" No definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" Equivocal osseous lesion at DV9/DV10 possibly of benign nature (Facet joint arthropathy) for CT correlation.

" The rest of the surveyed skeleton is free of any other definite active osseous lesions.

Much Obliged

Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====



Patient Name: Amani Mohamed Hussein Mohamed	Date : 10/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 56 Year(s)
Medical ID: 4303/2023	Pat. ID: 44487

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Neck :

- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilagenous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- Normal appearance of the neck muscles & vessels.
- Multiple variable sized small cervical lymph nodes the largest at right side measures 1.3 cm likely reactive for follow up
- Enlarged left thyroid lobe with a non enhanced hypodense lesion for us correlation.

OPINION:

Cervical lymphadenopathy with left thyroid lobe nodule for sonographic correlation.

Comparing to previous study done at 29/11/2023, the current study showing:

I-Chest:

- Both breasts are partially out of field.
- **Still noted** bilateral prominent axillary lymph nodes
- Clear both lung fields apart from lingula and lower lobes fine atelectatic bands
- **Still noted** Few subcentimetric right paratracheal and prevascular lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. Minimal pericardial effusion.
- Normal CT appearance of the thoracic aorta and the great vessels.

II-Abdomen and pelvis:



Patient Name: Amani Mohamed Hussein Mohamed	Date : 10/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 56 Year(s)
Medical ID: 4303/2023	Pat. ID: 44487

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Stationary course regard size and density of previously noted left adrenal heterogeneously enhancing mass lesion measures about 3.4x2.5 cm
- Still noted right (lateral limb) nodular thickening reaches about 7x6 mm.
- Still noted midline infra-umbilical fat containing hernia with defect measures 2 cm.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Left extra-renal pelvis and right lower pole calyceal simple cyst measures about 4x4 cm.
- Normal appearance of the pancreas, aorta and I.V.C
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- Still noted appendicular and axial skeleton shows innumerable sclerotic lesions. No vertebral structural collapse.

OPINION:

Stationary course since last study.

Dr/Reem Elwaly
Dr.Wafaa Ali, Msc

Dr. mohamed Mohsen, M.D.



Patient Name: Amani Mohamed Hussein Mohamed	Date : 02/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 56 Year(s)
Medical ID: 4303/2023	Pat. ID: 44487

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

➤ KNOWN CASE OF METASTATIC BREAST CANCER LAST STUDY DATE (10-08-2024)

- Part of the right breast out of field ,appeared part show distorted parenchma
- Still noted prominent axillary lymph nodes
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Still noted Subcentimeteric paratracheal and prevascular lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. right kidney show simple cortical cyst (4.5x4.5cm)
- Still noted stationary course regarding to size Right adrenal (lateral limb) nodular thickening about 7x6 mm.
- Still noted stationary course regarding to size Left adrenal mass heterogeneously enhancing mass lesion measures about 3.4x2.5 cm



Patient Name: Amani Mohamed Hussein Mohamed	Date : 02/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 56 Year(s)
Medical ID: 4303/2023	Pat. ID: 44487

- Still noted paraumbilical hernia ,defect 2cm with fat content
- Normal appearance of the pancreas, aorta, I.V.C
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton: still noted** innumerable sclerotic focal lesions all over axial and appendicular skeleton .

Opinion:

- **Stationary course comparison to last study**

Dr. Marwa Kamal Saber

Prof. Menna Tallah Hanafy, M.D.



Patient Name: Amani Mohamed Hussein Mohamed	Date : 29/11/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
Medical ID: 4303/2023	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast central area of architectural distortion and few right axillary level I lymph nodes, the largest about 11x6 mm.
- Clear both lung fields.
- Few sub cm right paratracheal and prevascular lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. Minimal pericardial effusion.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- Right adrenal (lateral limb) nodular thickening reaches about 7x6 mm.
- Left adrenal mass heterogeneously enhancing mass lesion measures about 3.4x2.5 cm
- Midline infra-umbilical fat containing hernia with defect measures 2 cm.
- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Left extra-renal pelvis and right lower calyceal simple cyst measures about 4x4 cm.
- Normal appearance of the pancreas, aorta and I.V.C



Patient Name: Amani Mohamed Hussein Mohamed	Date : 29/11/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
Medical ID: 4303/2023	

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The appendicular and axial skeleton seen heavily studded by innumerable sclerotic lesions. No vertebral structural collapse.

OPINION:

- ***Right breast parenchymal distortion and ipsilateral axillary lymph nodes.***
- ***Bilateral adrenal gland lesions likely metastases.***
- ***Extensive bony sclerotic deposits.***
- ***No definite hepatic or pulmonary metastatic deposits.***

Dr. Shaimaa Megahed, MSc

Dr. Amr Farouk, M.D.

PATIENT NAME: AMANI MOHAMMED HUSEN MOHAMMED	PATIENT ID #: 10123233754
GENDER: Female	ORDER #: 2539189,2539190
AGE: 55 y	DATE OF SERVICE: 18-Oct-2023 04:28 PM

Report: - all studies

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right palpable lump.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Right breast show an ill-defined speculated dense lesion at the centre of the breast extending to the retro-areolar area causing nipple retraction and architectural distortion, measures about 6.6 x 5 cm (at MLO view). Two rounded dense lesions at UOQ. A retro-areolar linear calcifications area seen.
- No left spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed:
- Right breast shows an ill-defined speculated hypoechoic lesion at the centre of the breast extending to the retro-areolar area causing nipple retraction, measures about .6 x 4 cm.
- Two subcentimetric anechoic lesions at UOQ 10 o'clock, measure about 4 x 3 mm and 8.3 x 4 mm? Simple cysts.
- No left solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex on left side.
- Indeterminate right axillary lymph node (1-2) (level I) with mild thick cortex up to 4.4 mm and preserved hilum.
- Right one supra-clavicular lymph node, measures collectively about 6 x 3 cm.
- Nonspecific left axillary lymph nodes.

OPINION:

- Right central malignant mass lesion (**BIRADS 5**), for histopathological correlation.

Recommendation - all studies

Referral to clinic

Referral to clinic

Reported By,

Dr, Samah Sayed,MSc

18-Oct-2023 05:57 PM